

Medio Pharm Analysis Report

Overall Hospital Operations

The hospital recorded 1K patients, 3K admissions, and 12K visits, indicating high overall activity. Emergency and Pulmonology handled the largest admission volumes, while flu, respiratory conditions, and hypertension were among the leading diagnoses. Medication-related concerns were the most common reason for visits.

The 6.63% readmission rate, along with 189 readmissions, 432 ICU cases, and 90 deaths, highlights areas requiring closer monitoring. Admissions also showed a significant decline, from 969 in February to 17 in July, warranting further investigation.

Department Performance & Readmissions

Readmissions account for 189 cases, with an 18.90% readmission rate and 755 flagged 30-day readmissions, highlighting the need for closer monitoring. General Medicine, Cardiology, and ICU recorded the highest readmission volumes, while asthma, heart disease, diabetes, and hypertension were prominent chronic conditions among repeat patients.

Readmissions peaked at 76 in February before declining through June. Response times remained consistent at about 16 minutes across departments, suggesting that patient outcomes—not response speed—may warrant greater attention. Although “Recovered” outcomes dominate, ICU and deceased cases remain important risk areas.

ER Load & Patient Vitals

The ER recorded 507 admissions, with diabetes and hypertension as leading chronic conditions. The data shows a very high number of abnormal vital readings, while average heart rate and SpO₂ were both 92.5. The average response time was 15.84 minutes, indicating potential room for faster triage. Although 375 patients recovered, ICU transfers, readmissions, and deaths remain important outcomes to monitor. ER admissions peaked at 159 in February before falling sharply to 2 in July, suggesting a possible seasonal or reporting pattern.

Recommended Actions

- Strengthen discharge planning and post-discharge follow-up to reduce avoidable readmissions.
- Prioritize staffing and resources for high-demand departments, especially Emergency, Pulmonology, Cardiology, and ICU.
- Expand chronic disease management for diabetes, hypertension, asthma, and cardiac conditions.
- Investigate the March–July admission decline, and February readmission spike to identify operational, seasonal, or data-related causes.
- Investigate the high rate of abnormal vital readings and strengthen monitoring of high-risk patients.
- Improve response times during peak periods and monitor ICU and readmission trends monthly.
- Standardize post-discharge monitoring hospital-wide, with particular attention to General Medicine, Cardiology, and ICU.